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Immunology
Competitive Intelligence Update

Brought to you by:
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FDA approved Sotyktu for active PsA

Company: BMS

Drug: Sotyktu (deucravacitinib, oral TYK2 inhibitor)

Date: March 6th, 2026

Source(s): Press release, FDA label

What happened:

- On its March 6th PDUFA date, the FDA approved Sotyktu (deucravacitinib, oral TYK2 inhibitor) for the treatment of adults with active psoriatic arthritis (PsA)
    - Approval is based on the positive results from the Phase 3 POETYK PsA-1 and POETYK PsA-2 studies
    - Approved dosage: 6mg once daily in adults
    - Consistent with the PsO label, Sotyktu is associated with the following warnings and precautions: hypersensitivity reactions, infections, tuberculosis, malignancy including lymphomas, rhabdomyolysis and elevated CPK, laboratory abnormalities, immunizations, and potential risks related to JAK inhibition
- The FDA label and HCP website have been updated; the patient website has not been updated yet to reflect the approval

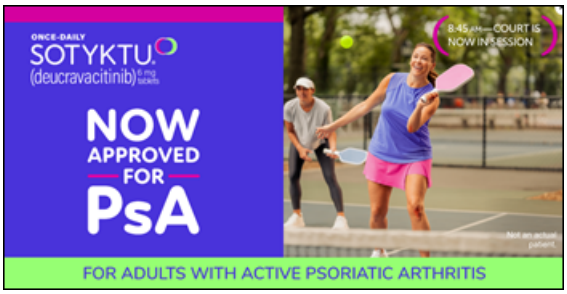
Impact (CI PoV): HIGH

- The FDA approval makes Sotyktu the first and only oral TYK2 inhibitor approved for adults with active PsA, expanding its 2022 approval in moderate-to-severe plaque psoriasis (PsO)
    - *This approval allows BMS to establish early physician familiarity and real-world evidence ahead of emerging oral competitors such as JNJ's*

Icotyde (icotrokinra, oral IL-23R antagonist) and Takeda's Zasocitinib (TAK-279, oral TYK2 inhibitor), with approvals expected in Q1 2028 and Q3 2028, respectively

Why it matters:

- The approval offers a new once-daily oral option that may appeal to patients seeking an alternative to injectable biologics while potentially providing a more manageable safety and monitoring profile than JAK inhibitors



Tell me more:

- FDA approval is supported by the Phase 3 multicenter, randomized, double-blind, placebo-controlled POETYK-PsA-1 and POETYK-PsA-2 trials that evaluated the safety and efficacy of Sotyktu in adults with active PsA
- **Study Design**
  - POETYK PsA-1 included 670 patients with active PsA who were bDMARD naïve
  - POETYK PsA-2 included 624 patients with active PsA who were bDMARD naïve or had previously received TNFα inhibitor treatment; the trial also included an apremilast safety reference arm
  - Primary endpoint: ACR20 at Week 16
- **Study Results**
  - Efficacy:

- Both trials met their primary endpoints, demonstrating sustained improvements across joint pain and swelling as well as psoriatic skin lesions

	PsA-1			PsA-2		
Endpoint	*Sotyktu* (N=336)	Placebo (N=334)	Difference from Placebo (95% CI)	*Sotyktu* (N=312)	Placebo (N=312)	Difference from Placebo (95% CI)
ACR20 response, %	54[b]	34	20(12, 27)	54[b]	39	15 (7, 23)
ACR50 response, %	24	14	11 (5, 17)	29	16	13 (6, 19)
ACR70 response, %	12	5	6 (2, 10)	10	5	5 (1, 9)
Minimal disease activity response, %	19[c*]	10	9 (4, 14)	26[c†]	15	11 (5, 17)

- Safety:
  - The overall safety profile in active PsA was generally consistent with the safety profile previously seen in PsO
  - Most common adverse reactions were upper respiratory infections, increased blood creatine phosphokinase, herpes simplex, mouth ulcers, folliculitis, and acne
- HRQoL:
  - Improvements were seen in SF-36 PCS score at Week 16 compared to placebo (key secondary endpoint)
  - Improvements were also reported across all four SF-36 PCS domain scale scores: physical functioning, role-physical, bodily pain, and general health

- **Outlook:**
  - Regulatory applications for Sotyktu are under review in the EU, Japan and China
  - **Pediatric PsA**: Phase 3 study ongoing; PCD: March 2030
  - **SLE:** Phase 3 POETYK-SLE-1 & POETYK-SLE-2 ongoing, with data expected in 2026; PCDs: October 2026;

- **SjD:** Phase 3 POETYK-SjS-1 ongoing, with data expected in 2027; PCD: November 2026

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Kyowa Kirin terminates all ongoing clinical programs for rocatinlimab, including AD

Company: Kyowa Kirin

Date: March 3rd, 2026

Drug: rocatinlimab (SC OX-40 inhibitor)

Source(s): Press release

What happened:

- Kyowa Kirin discontinued **all ongoing clinical trials of rocatinlimab across moderate-to-severe AD, PN, and asthma**
  - Decision followed a planned global safety review concluding that **risk may outweigh benefit**, driven by **malignancy concern**, including Kaposi's sarcoma (one new confirmed; one suspected) alongside a **previously confirmed and reported case, suggesting a potential link to OX40 modulation**

Impact (CI PoV): HIGH

- It seems unlikely that Kyowa Kirin will pursue rocatinlimab's regulatory submission in moderate-to-severe AD; after required safety follow up visits, the companies expect studies to be formally terminated

- This outcome adds to the existing concerns surrounding OX40-pathway modulation, reinforcing perceived risks related to immune-surveillance, and **raising the evidentiary bar for long-term safety characterization in adjacent programs**

Why it matters:

- Discontinuation of rocatinlimab would **remove one of the most advanced OX40-pathway assets from the moderate-to-severe AD landscape** and resets risk–benefit expectations for the class as immune-surveillance questions are re-tested
- For Sanofi's amlitelimab (SC OX-40L inhibitor), the broader pathway-level safety discussion could introduce incremental scrutiny into its near-term regulatory trajectory. CI will continue monitoring for any updates.

Tell me more:

- This update follows January 2026 update that Amgen had terminated its collaboration with Kyowa Kirin for rocatinlimab, returning full regulatory and commercialization responsibility to Kyowa Kirin
    - At that time, Kyowa Kirin had confirmed intent to file in the US in H1 2026 based on Phase 3 ROCKET data
- Although the overall number of malignancy events in the program was noted to remain below expected background rates, **the nature of the Kaposi's sarcoma cases introduces a plausible biological risk that cannot be excluded**, contributing to the unfavourable risk-benefit assessment
- Kyowa Kirin expressed notable disappointment, emphasizing that rocatinlimab had "*demonstrated durable and clinically meaningful efficacy in moderate-to-severe AD"* within the ROCKET program; however, management noted that safety profile had evolved, prompting a *"decisive and cautious"* action prioritizing patient safety
- **The company is notifying investigators and regulatory authorities;** participants will complete required safety follow-up, after which **trials will be formally terminated**
    - Kyowa Kirin and Amgen will conduct a comprehensive dataset analysis and provide further updates after assessments are complete

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Takeda Announced Positive Results from the Ph3 KEPLER Trial of Entyvio in Pediatric Patients with Moderately to Severely Active UC

Company: Takeda
Drug: Entyvio

Date: February 19, 2026
Source(s): press release

What happened:

- Takeda announced positive results from the pivotal Ph3 KEPLER trial of Entyvio (vedolizumab, α4β7 integrin inhibitor, **IV/IV**) in **pediatric patients aged 2 years and older with moderately to severely active ulcerative colitis**

- Results presented at the ECCO 2026 (abstract link)

Impact (CI PoV): MEDIUM

- The positive data strengthen Takeda's position in the IBD market and bolster its strategy to expand Entyvio's indications. With this approval, Takeda addresses a segment where therapeutic options remain limited.

- Takeda is moving toward regulatory filings in the US, EU, and other markets for Entyvio IV in pediatric patients aged 2–17 years with moderately to severely UC

Why it matters

- The KEPLER trial results represent a significant clinical milestone for Takeda, demonstrating that Entyvio can effectively induce and maintain clinical remission in pediatric patients as young as two years old. These results address a critical treatment gap for children and adolescents with moderate to severe UC
- Entyvio is currently positioned as the #1 prescribed brand in IBD in the US; due to its tolerable safety profile, Entyvio could be preferred over anti-TNFs currently approved in the pediatric population

Tell me more:

- The KEPLER study enrolled 120 pediatric patients aged 2 to 17 years with moderately to severely active UC, who had shown inadequate response to conventional treatments, including steroids and immunomodulators, and/or TNF antagonists
- Patients initially received open-label IV vedolizumab during a 14-week induction phase. At week 14, 93/ 120 patients achieved a clinical response, who were randomized into maintenance therapy groups
    - 47 patients were assigned to a low-dose vedolizumab regimen, and 46 patients were assigned to a high-dose vedolizumab regimen
- Among the 93 patients randomized to maintenance therapy,
    - 47.3% achieved the primary endpoint of clinical remission at week 54
    - 34.7% of patients reached clinical remission at week 14; 29% of participants achieved sustained clinical remission at both week 14 and week 54
- No new safety signals identified. The most common treatment-emergent adverse events (occurring in ≥10% of patients) were upper respiratory infection (30%), worsening of UC (17.5%), and pyrexia (12.5%)
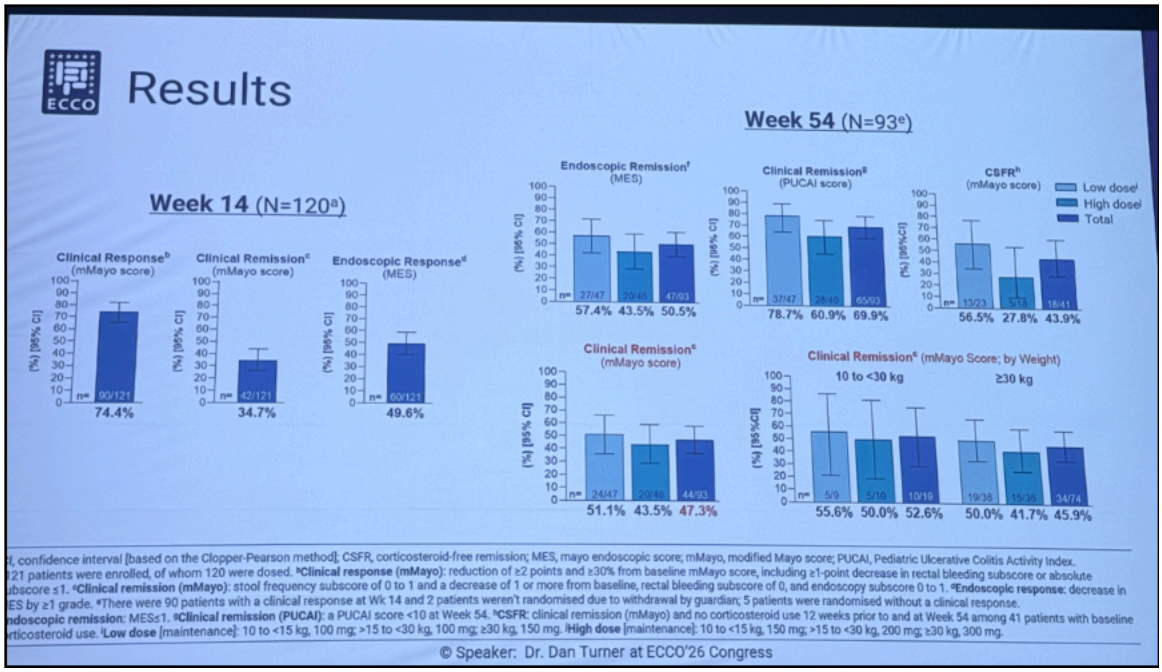
Image from the data presented at ECCO 2026